Case Report

LC 4 - Five-Component Submission

VERSION	DATE	AUDIENCE
v1.0	2026-09-01	Learner submission

Submission structure: Complete five components. There is no separate overall case-summary section. Use minimum necessary synthetic information and cite the supplied evidence documents.

Component 1 - System Context and Dependency Analysis

<table>
<tr><th>Field</th><th>Learner entry</th></tr>
<tr><td>System context</td><td>
INTEGRATION PICTURE
Systems are named exactly as in the Fact Sheet's System boundary table. “Direct technical integration” is marked Yes only where the Fact Sheet documents an automated connection. Anything that depends on Dr. Chen acting is marked No (human-mediated). The Fact Sheet has no section numbers, so citations name the table and row (for example, Fact Sheet › Policy baseline › External sharing).
<table>
<tr><th>System / service</th><th>Type</th><th>Role in this workflow (evidence)</th><th>Direct technical integration?</th></tr>
<tr><td>Epic Hyperspace</td><td>Clinical application</td><td>Upstream clinical context only. The case summary is not pulled from Epic here: the synthetic file is already in the SharePoint team site.
Fact Sheet › System boundary › Epic Hyperspace; Case Packet › File, Collaboration location</td><td>No. “No direct Epic-to-Copilot integration is established.” No Epic link to SharePoint or Outlook is documented either.</td></tr>
<tr><td>Microsoft Entra ID</td><td>Microsoft cloud identity service (formerly Azure Active Directory)</td><td>Signs in Dr. Chen's workforce account for SharePoint / OneDrive, Microsoft 365 Copilot and Outlook. Evidence it can give: user identity, group membership, authentication methods, Conditional Access results.
Fact Sheet › System boundary › Microsoft Entra ID</td><td>Yes, for sign-in only. Every Microsoft 365 surface in this workflow depends on Entra ID authentication. No case content passes through it.</td></tr>
<tr><td>SharePoint / OneDrive</td><td>Microsoft 365 file collaboration</td><td>Holds Synthetic Case Summary – Patient A.docx in the approved SharePoint team site library. It enforces who can open the file and shows the access list in Manage Access. Dr. Chen calls it “OneDrive”; the Fact Sheet and Case Packet place the file in the SharePoint team site, reached through the Microsoft 365 collaboration experience.
Fact Sheet › System boundary › SharePoint / OneDrive; Policy baseline › External sharing</td><td>No (human-mediated). Dr. Chen picks the file, the recipient and the link type. SharePoint enforces what she sets, but nothing moves the file to Copilot or Outlook.</td></tr>
<tr><td>Microsoft 365 Copilot</td><td>Microsoft 365 AI drafting service</td><td>Drafts the administrative email from typed synthetic facts only. It is not given the case file.
Fact Sheet › System boundary › Microsoft 365 Copilot; Policy baseline › Copilot use, Input data</td><td>No (human-mediated). Dr. Chen types the prompt and moves the draft into the email. No Epic connection.</td></tr>
</table>
</td></tr>
</table>

Outlook / Exchange Online	Microsoft 365 messaging	Outlook is where Dr. Chen writes, reviews and sends; Exchange Online delivers. She owns recipient selection, message review and Send. *Fact Sheet › System boundary › Outlook / Exchange Online*	**No (human-mediated).** Delivery runs automatically only after Dr. Chen selects Send.
Coursera AI	Simulation participant, outside the tenant	Plays Dr. Chen in the coaching role play, with synthetic dialogue only. *Fact Sheet › System boundary › Coursera AI*	**No.** "Not connected to the Contoso tenant or clinical systems."

WORKFLOW STEPS: HUMAN-MEDIATED OR TECHNICAL

#	Workflow action	Done by	Classification	Evidence
1	Confirm the synthetic case summary is in the approved team site library. No Epic export.	Dr. Chen	Human-mediated	Case Packet › File, Collaboration location; Fact Sheet › SharePoint / OneDrive
2	Sign in to SharePoint, Copilot and Outlook.	Dr. Chen → Entra ID	**Technical integration** (authentication)	Fact Sheet › Microsoft Entra ID
3	Confirm the colleague is the authenticated person named in the approved request.	Dr. Chen	Human-mediated (clinician decision)	Case Packet › Recipient; Fact Sheet › Outlook / Exchange Online; KB-410 bullet 3
4	Share with that person only: recipient-specific, Can view, no Anyone link.	Dr. Chen (Tier 1 explains)	Human-mediated; SharePoint enforces the permission	Fact Sheet › Policy baseline › External sharing, Permissions
5	Read the Manage Access list.	Dr. Chen (Tier 1 asks what she sees)	Human-mediated	Fact Sheet › External sharing ("Manage Access evidence and recipient confirmation")
6	Enter the RCCOF prompt card (synthetic administrative facts only).	Dr. Chen	Human-mediated	Fact Sheet › Copilot use, Input data
7	Copilot returns a draft.	Microsoft 365 Copilot	Technical, inside Copilot only; no onward data flow	Fact Sheet › Microsoft 365 Copilot
8	Check the draft against the Case Packet and the KB.	Dr. Chen (Tier 1 coaches)	Human-mediated	Fact Sheet › Prompt verification
9	Add recipient, link and signature, then select Send.	Dr. Chen	Human-mediated; Exchange Online delivers after Send	Fact Sheet › Outlook / Exchange Online; Tier 1 authority

Dependency chain

Traced in the Analysis Worksheet order (§1.1): identity first, because an identity fault looks like a problem in any of the other three. Each row answers the required question for that category.

Category · service	Normal function (what must work)	If degraded or down: what breaks, for whom, in what order	Human vs. technical · side (Worksheet §1.2)
1 · Identity Microsoft Entra ID	**Entra ID authenticates Dr. Chen** for SharePoint / OneDrive and is the identity boundary for Copilot and Outlook (Conditional Access applies). **The colleague must authenticate as the named recipient** before the file opens (authenticated recipient-specific sharing). *Fact Sheet › Entra ID; External sharing*	If Dr. Chen can't sign in or Conditional Access blocks her, the workflow stops at step 2: no file, no Copilot, no Outlook. If the colleague can't authenticate, the email still arrives but the file won't open, so the consult starts without the summary. The fix is never an Anyone link. **Unable to Verify:** the Fact Sheet doesn't describe how an external	Technical integration (sign-in). Recipient choice is human. **Upstream of both**

			colleague signs in, or Dr. Chen's Copilot license. Record it and escalate if blocked (Fact Sheet › Decision boundaries, bullet 4).	
	2 · Messaging Outlook / Exchange Online	**Exchange Online delivers** Dr. Chen's email, with the pasted link, to the address in the approved request. **Downstream dependency for the colleague:** the email is only a pointer. Opening the file still depends on Identity (the colleague signs in) and File access (permission granted to that same identity). *Fact Sheet › Outlook / Exchange Online*	If delivery is delayed or bounces, the colleague has no pointer before the consult. The sharing permission itself is unaffected. A typo in the To line sends the pointer to the wrong person; the link should still not open for them, because access is tied to the named recipient. Tier 1 does not resend on Dr. Chen's behalf.	Human-mediated: Dr. Chen enters the address and sends. **Microsoft 365 side**
	3 · File access SharePoint / OneDrive	**SharePoint / OneDrive decides whether the colleague can open the file.** Required configuration for external collaboration: authenticated recipient-specific sharing (People you choose / Specific people); no anonymous or Anyone link; no broadening of site or library permissions. Can view is the minimum-necessary level. The Fact Sheet sets no level, so Dr. Chen confirms it. *Fact Sheet › External sharing, Permissions; KB-410 bullets 1–2*	If recipient-specific external sharing is blocked by tenant policy or inherited permissions, the approved path is unavailable: stop and escalate. Don't switch the link type (Fact Sheet › Permissions). If it is misconfigured (Anyone link, wrong person), the file may be disclosed inappropriately: privacy route. If the service is degraded, the colleague can't open the file even when the email arrives.	Human-mediated setting; SharePoint enforces it. **Microsoft 365 side**
	4 · Clinical system availability Epic Hyperspace	**No runtime role.** Epic is upstream context only. **The case summary was produced independently:** a synthetic, role-coded narrative written for the exercise and already stored in SharePoint. It is not stored in or exported from Epic in this workflow. *Fact Sheet › Epic Hyperspace; Case Packet › File, Synthetic source summary*	An Epic outage does not stop sharing, drafting or sending; it affects Dr. Chen's wider clinical work, which is outside this request. If she reports an Epic problem, Tier 1 names the category and warm-transfers without diagnosing it (Worksheet §1.4).	No integration. **Clinical application side**

> **Failure sequence in one line**
>
> An identity failure stops everything. A file-access failure leaves an email pointing to a file nobody can open. A messaging failure leaves a correctly shared file with no pointer. An Epic failure doesn't touch this workflow.
>
> **Not one of the four categories:** Microsoft 365 Copilot is off the critical path. If it is unavailable, Dr. Chen writes the short email herself. The 45-minute deadline does not justify any workaround (Case Packet › Deadline; ITS-STD-014 §4 NA-03 boundary note).

Human vs. technical boundary

The only technical integration documented is Entra ID sign-in. Every movement of content between systems is human-mediated. Dr. Chen decides what goes into Copilot, moves the draft into Outlook, chooses the recipient, sets the share and selects Send. No Epic-to-Copilot or Epic-to-Microsoft 365 integration is assumed.

WHO OWNS EACH DECISION

Decision	Owner	Tier 1's part	Evidence
Clinical content; whether	Dr. Chen	None. Does not judge clinical	Fact Sheet › Decision

to consult		appropriateness.	boundaries, bullet 1
Recipient selection and authorization	Dr. Chen	Explains the check. Escalates if the recipient can't be authenticated.	Fact Sheet › Outlook / Exchange Online; KB-410 bullet 3; KB-422 bullet 3
Disclosure decision	Dr. Chen (doubts go to the Privacy Office)	Is not the disclosure approver.	Fact Sheet › Tier 1 authority; Decision boundaries, bullet 2
Sharing configuration	Dr. Chen	Explains Manage Access and records the evidence. Never broadens access.	Fact Sheet › External sharing, Permissions; KB-410 bullet 4
Copilot prompt and draft review	Dr. Chen	Supplies the prompt card and verification plan; coaches.	Fact Sheet › Copilot use, Prompt verification
Send	Dr. Chen	Never sends.	Fact Sheet › Tier 1 authority; KB-422 bullet 4

TIER 1 CHECKS AND ESCALATION TRIGGERS

Dependency point	Tier 1 can check (guided, read-only)	Escalation trigger	Warm-transfer target (Worksheet §1.3)
Identity	Dr. Chen confirms she is signed in with her own workforce account and can open SharePoint, Copilot and Outlook. Tier 1 records any Conditional Access message she reads out.	Sign-in blocked; Conditional Access denial; account or group-membership problem; colleague can't authenticate as the named recipient.	Identity Administrators
File access: setup	Dr. Chen opens Share, then Manage Access, and reads out the link type, the named person and the permission.	Recipient-specific option missing, or blocked by tenant policy; inherited permissions prevent the approved path; a fix would need wider access.	M365 Admin Team, with a ticket note (Fact Sheet › Permissions)
File access: exposure	The same Manage Access read-out.	An Anyone link, or someone unexpected already has access: possible inappropriate disclosure.	Privacy Office intake (the organization-designated privacy / security route), with minimum-necessary evidence (Fact Sheet › Privacy escalation)
Messaging	Outlook opens. Dr. Chen reads the To line against the approved request.	Bounce or delivery-failure notice; Outlook unavailable.	M365 Admin Team
Copilot	Copilot appears in her approved experience. The prompt passes Checklist C1–C11.	Copilot missing because of role, license, tenant policy or product state: record Unable to Verify (Fact Sheet › Decision boundaries, bullet 4). Use-case boundary unclear.	M365 Admin Team (availability); team lead flag (ITS-STD-014 §6.3)
Clinical system	Name the category only.	Epic unavailable, or a fault inside Epic.	Epic Analyst team, without diagnosis (Worksheet §1.4)
Platform	Nothing to check at Tier 1.	A service-wide fault beneath the tenant (hosting or platform).	Azure Engineering

The Worksheet names receiving teams rather than tier numbers; every target sits above Tier 1 (Tier 2 or 3). Tier 1 hands off with the documented chain and does not assign ownership or priority (Worksheet › Scope check).

Evidence sources

- **Technical Environment and Contoso Policy Fact Sheet v1.0:** System boundary (all six rows); Policy baseline (Copilot use, Input data, External sharing, Permissions, Tier 1 authority, Privacy

	escalation); Decision boundaries 1–4. • **Synthetic Case Packet and Source Summary v1.0:** Source facts (File, Collaboration location, Recipient, Deadline, Tier 1 boundary); Stop condition; Synthetic source summary. • **Dependency Analysis Worksheet ITS-TRN-016-B:** §1.1 category order, §1.2 boundary sides, §1.3 warm-transfer targets, §1.4 Epic-side limit, Scope check. Its CASE-01 to CASE-03 grids belong to another dataset and were not used. • **System-Context Mapping Template:** Sections 1–4 structure used. Its sample rows were not reused: they cite Fact Sheet section numbers that don't exist, assume an Epic export and upload step, and name escalation groups that appear in none of the supplied documents. • **Mock Internal KB:** KB-410 bullets 1–4 and KB-422 bullets 3–4, for the sharing and send boundaries.

Component 2 - Copilot Use Case Determination

<table>
<tr><th>Field</th><th>Learner entry</th></tr>
<tr><td>Request element-by-element classification</td><td>Dr. Chen's request is split into four elements, and each is assessed on its own. The request is not classified as a single unit: ITS-STD-014 §4 makes each not-approved category independently disqualifying, and the RCCOF Template §1.1 (SCN-002 note) shows one request splitting across the line.
<table>
<tr><th>Request element</th><th>Classification</th><th>Policy basis (cited section)</th></tr>
<tr><td>Administrative email drafting</td><td>APPROVED
Dr. Chen prompts; the draft is reviewed before use.</td><td>ITS-STD-014 §3 UC-02 User communication drafting (approved output: “a draft email … always reviewed and edited … before sending”), with §5 redaction and §6.1 human review. Exercise-specific approval: KB-431 bullet 1 (“administrative drafting from supplied synthetic notes or facts”); Fact Sheet › Copilot use.</td></tr>
<tr><td>Clinical case summarization</td><td>NOT APPROVED
Not attempted with Copilot.</td><td>ITS-STD-014 §4 NA-01 PHI-adjacent content (“a request to explain, interpret, paraphrase, or check clinical content”); §3 UC-02 Explicitly excluded (“Clinical instruction or guidance of any kind”); §4.4 test. Also KB-431 bullet 2; Fact Sheet › Copilot use.</td></tr>
<tr><td>Recipient authorization</td><td>ESCALATE
Not a Copilot task. Decided by Dr. Chen through the approved request; escalated if it can't be confirmed.</td><td>ITS-STD-014 §4 NA-02 (a determination of “whether a disclosure was permitted”); §2 (no autonomous determinations on privacy matters); §4 NA-03. Also KB-431 bullet 2 (“recipient authorization decisions” not approved); KB-410 bullet 3; KB-422 bullet 3; Checklist C8.</td></tr>
<tr><td>Disclosure decisions</td><td>ESCALATE
Clinician-owned. Any doubt about whether the disclosure is permitted goes to the Privacy Office.</td><td>ITS-STD-014 §4 NA-02; §2; §4 NA-01 “What to do instead” (possible improper disclosure → Privacy Office intake). Also KB-410 bullet 3; Fact Sheet › Copilot use (“no … autonomous disclosure decisions”), Tier 1 authority, Privacy escalation.</td></tr>
</table>
Not Approved means the element is simply not done with Copilot; no one else has to decide anything. Escalate means a decision still has to be made, by someone other than Copilot or Tier 1.</td></tr>
<tr><td>Approved / not approved / escalate decision</td><td>Split decision. Copilot is approved for element 1 only.
• Element 1, Approved: Dr. Chen uses Copilot herself, with the Component 3 prompt card, after the file is shared. Tier 1 does not run Copilot for her.
• Element 2, Not Approved: no clinical content enters Copilot, and the email contains none. The case summary already exists; the email only points to it.
• Elements 3 and 4, Escalate: they stay with Dr. Chen through the approved, human-controlled workflow (approved request, recipient-specific sharing, Manage Access). If the recipient can't be authenticated or the sharing route is blocked, go to Identity Administrators or the M365 Admin Team. If there is any question whether the disclosure is permitted, go to Privacy Office intake. If the boundary itself is unclear, flag it to the team lead (ITS-STD-014 §6.3).</td></tr>
</table>

	• **The 45-minute deadline changes none of this** (Case Packet › Deadline; ITS-STD-014 §2 and NA-03 boundary note: urgency does not create an exception).
Approved-use-case citation	• **Primary:** ITS-STD-014 v3.2 §3 **UC-02** User communication drafting. Approved output: a draft email, reviewed and edited before sending. Required controls: redact per §5; the author of record is responsible for accuracy, tone and completeness. • **Exercise-specific approval: KB-431 bullet 1**, and Fact Sheet › Policy baseline › Copilot use ("Administrative drafting only"). Fact Sheet › System boundary › Microsoft 365 Copilot: "Permitted only for the supplied administrative drafting use case and only with synthetic or otherwise approved data." • **Fit note:** UC-02 is written for service-desk messages to end users, but here the author is Dr. Chen writing to a colleague. KB-431 bullet 1 matches the task exactly, so UC-02 is applied for its controls together with KB-431, not stretched as a closest fit (RCCOF Template §1, gate row 1). • **Document note for the team lead:** ITS-STD-014's header names Contoso but its banner names Northridge, and its next review date (6 July 2026) has passed. This is recorded, not resolved: interpreting the standard is outside Tier 1 scope (§2).
Rationale	**ADMINISTRATIVE EMAIL DRAFTING: APPROVED** A short, non-binding cover note built from supplied synthetic facts is exactly what KB-431 bullet 1 approves and what UC-02 permits as a reviewed draft. It needs no clinical content, so the §4.4 test is passed: removing clinical detail still leaves a coherent task. **CLINICAL CASE SUMMARIZATION: NOT APPROVED** Summarizing the case would have Copilot paraphrase or interpret clinical content. ITS-STD-014 §4 NA-01 excludes any task "whose substance is clinical," including requests to "explain, interpret, paraphrase, or check clinical content, however routine it appears," and UC-02 excludes "clinical instruction or guidance of any kind." The §4.4 test confirms it: the clinical narrative is the subject, not incidental, and a clinician would have to check the output. Removing names and dates does not change this, because redaction "does not convert a clinical question into a technical one" (NA-01). KB-431 bullet 2 and Fact Sheet › Copilot use add that diagnosis, clinical decision-making and interpretation of patient data are out of scope. Nothing is lost: the summary is already written, and Dr. Chen keeps the clinical reasoning for the consult. Any clinical question put to Tier 1 goes to the clinical informatics on-call (NA-01 "What to do instead"). **RECIPIENT AUTHORIZATION: ESCALATE** Deciding that this partner colleague may receive the file is an authorization decision with privacy consequences. KB-431 bullet 2 lists "recipient authorization decisions" as not approved for Copilot. ITS-STD-014 §4 NA-02 covers any determination of "whether a disclosure was permitted," and §2 puts such determinations outside Tier 1 as well. So neither Copilot nor Tier 1 makes this call. Dr. Chen confirms the colleague is the authenticated person "designated in the approved request" (Case Packet › Recipient; KB-422 bullet 3; KB-410 bullet 3). Tier 1's part is to explain the Manage Access check, and to escalate if the recipient can't be authenticated or the recipient-specific route is blocked (Case Packet › Stop condition; Fact Sheet › Permissions). **DISCLOSURE DECISIONS: ESCALATE** Whether to share, what to share and at what access level stay with the clinician. Copilot may not make "autonomous disclosure decisions" (Fact Sheet › Copilot use), and Tier 1 "does not act as the disclosure approver" (Fact Sheet › Decision boundaries, bullet 2; Tier 1 authority). Whether a particular disclosure is permitted is a privacy determination under ITS-STD-014 §4 NA-02 and §2. Any doubt goes to the organization-designated privacy route (Privacy Office intake) with minimum-necessary evidence (Fact Sheet › Privacy escalation). A correct link setting does not settle the question: a secure configuration "does not by itself establish HIPAA compliance" (Fact Sheet › Decision boundaries, bullet 3). Sending on the strength of Copilot output without human review would also be NA-03.

Component 3 - Prompt Design and Verification

Field	Learner entry
RCCOF prompt	For **element 1 only**. This is a prompt card for Dr. Chen to enter herself after the file is shared; Tier 1 does not run it. It follows the RCCOF Template (ITS-TRN-014-C §2) and uses only Case Packet facts.

PROMPT CARD: administrative cover email

ROLE

Act as an administrative communications assistant for a family medicine physician. Write in a professional, concise and collegial register for a physician reader at a partner clinic. Assume the reader knows what a curbside consultation is. Do not assume the reader knows where the case summary is or how to open it.

CONTEXT

Request: draft a short introductory email from the physician to a partner-clinic colleague before a curbside consultation. Facts you may use (complete list):

1. The sender is a family medicine physician.
2. The reader is a clinician at a partner clinic, named in the physician's approved request.
3. The purpose is an upcoming curbside consultation about a synthetic case summary.
4. The synthetic case summary has been shared with the reader only, from an approved SharePoint team site.
5. The physician wants the reader to review the summary before the consultation.

Use nothing else.

CONSTRAINTS

- Use only the five facts above. Do not add facts, dates, times or details.
- Do not include clinical details, symptoms, interpretation or opinion about the case.
- Do not include patient names, identifiers, dates or locations.
- Do not name or invent the recipient. Write [RECIPIENT NAME].
- Do not write a link, file name or file location. Write [LINK].
- Do not state that the reader is authorized, or that sharing is approved or compliant.
- If a detail you need is not in the facts, write UNKNOWN - VERIFY.
- Draft only. Do not send.

OUTPUT FORMAT

1. Subject line, under 10 words.
2. Greeting: “Dear [RECIPIENT NAME],”
3. Body, 2–4 sentences: the purpose of the consultation; the summary is available at [LINK]; please review it before the consultation.
4. Close with [SIGNATURE].
5. Under 120 words. No preamble, notes or alternative versions.

Design choices: the [RECIPIENT NAME] and [LINK] placeholders remove the two weaknesses found in the Mock Output (a generic recipient and an unverifiable link claim). The UNKNOWN - VERIFY line stops gaps coming back as invented details (RCCOF Template §2.1). The file name, the recipient's email and the consult time are left out as not needed.

AI input eligibility check

Completed before the prompt was written. **Pass** means confirmed from the supplied documents. **Clinician check** means Dr. Chen confirms it at the guide step shown; Tier 1 does not tick it for her.

#	Checkpoint	Status	Evidence / action
C1	Synthetic source confirmed	**Pass**	Only Case Packet “Source facts” and “Administrative email facts” used.
C2	No real identifiers	**Pass**	See the identifier table below. Recipient name, email and file name are kept out of the prompt.
C3	No secrets	**Pass**	No passwords, codes, tokens or keys. Dr. Chen never reads a sign-in code to Tier 1.
C4	Permitted Copilot task	**Pass**, element 1 only	Component 2; KB-431 bullet 1; ITS-STD-014 §3 UC-02.
C5	Approved surface	**Pass** (assumed per activity); live check at guide step 11	Fact Sheet › Microsoft 365 Copilot. If Copilot is missing: Unable to Verify, then escalate (Decision boundaries, bullet 4).
C6	Source verification ready	**Pass**	Case Packet › Synthetic source summary open and identifiable.

C7	Policy verification ready	**Pass**	KB-410, KB-422, KB-431; ITS-STD-014.
C8	External recipient check	**Clinician check**, guide steps 6, 16 and 19	Dr. Chen matches the colleague to the approved request. Stop if not named.
C9	Sharing control check	**Clinician check**, guide steps 3–9	People you choose, Can view, no Anyone link; Manage Access read-out.
C10	Human final review	**Clinician check**, “Check before you send”	Dr. Chen reviews the output, recipient, link and final message before Send.
C11	Stop condition review	**Pass**	Stop list in the guide and Component 2; the deadline does not override it.

IDENTIFIER CATEGORIES CHECKED AND ACTION TAKEN (ITS-STD-014 §5.1; CHECKLIST C2–C3)

Identifier category	In the material for the prompt?	Action taken
Patient name	No. The source uses the role label “Patient A” only.	Prompt makes no reference to the patient.
MRN; account, encounter or claim number; SSN; health-plan member ID	No. The Case Packet says the summary excludes MRNs and direct identifiers.	None needed; none in the prompt.
Date of birth; any date more specific than a year	No. The summary excludes exact dates.	Consult time also left out; the email says “upcoming.”
Street address, postal code; room, bed or unit	No.	None in the prompt.
Phone, fax, email, personal URL	Yes: the recipient's email is in the approved request.	Kept out of the prompt. Dr. Chen pastes it into the Outlook To line.
Device serial; IP; account IDs; tenant, site or library URL	No.	Location described only as “an approved SharePoint team site.”
Images or screenshots	No.	The case file is not attached to or referenced in Copilot.
Diagnosis, procedure, medication, result (clinical detail)	Yes: a non-specific symptom narrative in the case summary.	Excluded entirely; not needed for an administrative email (NA-01; KB-422 bullet 2).
Provider or clinician names	Yes: the recipient's name and Dr. Chen's name.	Replaced with role labels: [RECIPIENT NAME], “family medicine physician,” [SIGNATURE].
File name	Yes: “Synthetic Case Summary – Patient A.docx.”	Replaced with [LINK]; not typed into the prompt.
Secrets: passwords, tokens, keys, sign-in codes, security answers	No.	None.

Required note

This checklist is a **course training safety and data-minimization check**. It is **not a HIPAA de-identification determination** and does not replace organizational privacy or security review (Checklist › Purpose; Fact Sheet › Decision boundaries, bullet 3).

Source-fact verification

Path 1: factual accuracy. The Fixed Mock Copilot Output was checked line by line against the **Synthetic Case Packet and Source Summary**. Outcomes: Accurate / Inaccurate / Uncertain.

#	Output element (quoted)	Claim checked	Case Packet source	Outcome
1	Subject: “Curbside consultation - synthetic case summary”	Consult type; what is shared	Clinical context; File	**Accurate**

	2	“Hi Colleague at Partner Clinic,”	Who the reader is	Recipient: “Authenticated partner-clinic colleague designated in the approved request”	**Uncertain.** The role is right, but the named, authenticated colleague isn't identified. Replace it with the name from the request.
	3	“ahead of our curbside consultation”	Purpose and timing	Clinical context; Administrative email facts (“purpose of the consult”)	**Accurate**
	4	“to share the synthetic case summary we will review”	What is being shared	File; Administrative email facts	**Accurate**
	5	“The approved file is available through the authenticated sharing link prepared for this consultation.”	That an authenticated share already exists	Administrative email facts (“point the colleague to the authorized shared file”)	**Uncertain.** The packet says the file should be shared by the authorized route, but it can't show the share exists. True only after the Manage Access check (see Path 2).
	6	“Please review the summary before our discussion … let me know if you need anything”	Request and courtesy	Administrative email facts	**Accurate.** No new fact added.
	7	Signature: “Dr. Chen / Family Medicine / Contoso Health”	Sender identity	Clinician; Case brief (“family medicine physician at Contoso Health”)	**Accurate**, but it wasn't in the input context. Dr. Chen confirms her usual signature.
	8	Whole draft: omissions	No clinical detail, identifiers, dates or extra facts	Synthetic source summary; Administrative email facts (“should not add facts”)	**Accurate**
	9	Clinical accuracy	n/a	No clinical content requested or present	**Not assessed.** Copilot gets no credit for clinical accuracy (Mock Output, assessment flag 4).

Policy / KB verification

Path 2: workflow and policy claims. The same output was checked against the **Mock Internal Knowledge Base Article Set**. Outcomes: Corroborated / Contradicted / Not addressed.

#	Claim or behavior in the output	KB article checked	Outcome	What Dr. Chen does
1	“authenticated sharing link” (workflow claim)	**KB-410** bullets 1–2: authenticated recipient-specific sharing; no Anyone links. Fact Sheet › External sharing.	**Corroborated** as the approved pattern. But the output can't prove the link exists, and “authenticated” is less specific than “recipient-specific.”	Keep the sentence only after Manage Access shows the colleague by name, with no Anyone link.
2	“The approved file … prepared for this consultation” (status claim)	**KB-431** bullets 2 and 4 (no disclosure decisions; check workflow statements); KB-410 bullet 3	**Not addressed.** No KB article can confirm that this file was approved for this recipient; only Dr. Chen's approved request can.	Confirms it is true before sending, or deletes “approved.”
3	Generic recipient: “Colleague at Partner Clinic”	**KB-422** bullet 3: use the authenticated identity from the approved request; do not invent a recipient	**Contradicted.** A generic stand-in is used instead of the authenticated identity.	Replaces it with the colleague named in the request.

	4	Recipient authorization	**KB-431** bullet 2; KB-410 bullet 3	**Not addressed**, correctly: the draft makes no authorization claim (Mock Output, flag 2).	Keeps the recipient check (guide steps 6, 16 and 19).
	5	Scope and tone: concise, no clinical detail	**KB-422** bullets 1–2; KB-431 bullet 2	**Corroborated**	Nothing further.
	6	Draft only, not sent; needs human review	**KB-431** bullet 3; KB-422 bullet 4	**Corroborated.** The output is a draft only.	Reviews and sends it herself.
Final verification decision	**Decision: usable as a draft after clinician edits. Not to be sent as-is.** No inaccurate facts and no clinical content were found. Two items need Dr. Chen's action before Send: **1.** Replace the generic greeting with the authenticated colleague named in the approved request (KB-422 bullet 3). **2.** Keep the "authenticated sharing link" sentence only after Manage Access shows that colleague with Can view and no Anyone link, and paste the link herself (KB-410 bullets 1–2). Confirm "approved" is true for her request, or remove it. She also confirms the signature block. Final review and Send stay with Dr. Chen (KB-422 bullet 4; KB-431 bullet 3). The output triggers no stop condition, because it contains no unsupported fact that can't be resolved against the sources (Checklist › Stop and escalate when). The Component 3 prompt card removes both weaknesses at the source.				

Component 4 - End-User Guide

Field	Learner entry
Title	**Sharing a case summary with a partner-clinic colleague and drafting the cover email** *For Dr. Chen · Before a curbside consult · About 20 minutes*
Prerequisites	**Before you start.** If anything is missing, stop and call the IT support desk (the technology help line). • **Account:** your own Contoso work account, signed in. • **Permission:** you can share files in the approved team site holding *Synthetic Case Summary – Patient A* (its **Share** button works). • **Tools:** Outlook and Microsoft 365 Copilot (the writing assistant in Outlook). • **To hand:** your approved consult request (colleague's name and work email), the case packet, and IT support's prompt card (the exact text for Copilot). • **Time:** about 20 minutes. The deadline never overrides a stop. *Button names may differ slightly. If one doesn't match, stop and call.*
Numbered steps	**PART 1 — GIVE YOUR COLLEAGUE ACCESS TO THE FILE** **1.** Open the approved team site's document library. → *You see Synthetic Case Summary – Patient A.* **2.** Select **Share** on that file. → *The Share window opens.* **⚠ PRIVACY CHECK — before you give anyone access** • Share only with the colleague named in your approved request. • The link setting must be **People you choose** (or **Specific people**). Never **Anyone**. **Stop if** they aren't named in the request, or **People you choose** is missing. Never pick a wider setting to make it work. **3.** Open the link settings (gear icon). → *Who-can-open choices appear.* **4.** Choose **People you choose**. → *That option is selected.*

	5. Set the permission to **Can view**. → *It reads Can view.* **6.** Paste your colleague's work email from the request. → *Their name appears. If it doesn't, stop.* **7.** Select **Copy link**. → *"Link copied" appears.* **8.** Open **Manage access** for the file (the list of everyone who can open it). → *The access list appears.* **9.** Read the access list. → *Only your colleague, with Can view. No Anyone link. Anything else: stop.* **PART 2 — DRAFT THE EMAIL WITH COPILOT** **⚠ PRIVACY CHECK — before you type into Copilot** • Paste only the prompt card. It holds no patient details. • Don't paste, attach or point Copilot to the case summary. • Don't add names, dates, symptoms or email addresses. **Stop if** the email seems to need clinical detail. That stays with you, not Copilot. **10.** Open a new email in Outlook. → *A blank message opens.* **11.** Select **Draft with Copilot**. → *A Copilot box opens.* **12.** Paste the prompt card text. → *It appears in the box.* **13.** Select **Generate**. → *A draft appears with gaps: [RECIPIENT NAME], [LINK], [SIGNATURE].* **14.** Select **Keep it**. → *The draft moves into your email.* **PART 3 — CHECK THE DRAFT, THEN SEND IT YOURSELF** **15.** Compare each sentence with the case packet. → *Each matches, with nothing added. If one doesn't, delete it.* **16.** Replace [RECIPIENT NAME] with your colleague's name from the request. → *The greeting names the right person.* **17.** Paste the copied link over [LINK]. → *The link shows in the email.* **18.** Replace [SIGNATURE] with your usual signature. → *Your name appears.* **19.** Paste your colleague's work email into the **To** line. → *It matches the request and Manage access.* **20.** Do the check below. → *All three checks pass.* **21.** Select **Send**. → *The email appears in Sent Items.*
Privacy-aware callout	Two privacy checks sit **inside the steps above**, in amber boxes, each **immediately before the action it governs**: • **Before you give anyone access:** placed just before step 3, where the link type and recipient are chosen. This is the sharing decision. It names what to look at (the approved request; the link setting) and what stops the task. • **Before you type into Copilot:** placed just before step 10, the first point where text could enter Copilot. This is the patient-detail decision. It names what may and may not be typed. • The send action is guarded by the "Check before you send" box in the next row.
Verification step	**✔ Check before you send (this check can fail)** All three must match: 1. **Your request:** the To line and the greeting name the same colleague. 2. **Manage access:** that colleague, Can view, no Anyone link. 3. **Case packet:** every sentence is supported; nothing clinical added. **If any check fails:** don't send, and don't re-share more widely. Leave the draft unsent and call the IT support desk. **Also stop and call if** • Manage access shows someone you didn't add, or an Anyone link. Call straight away. • Your colleague says the link won't open. • You're unsure whether this summary may be shared. • Copilot isn't available. You may write the email yourself.

<table>
<tr><td></td><td>What happens when you call. Say which step you stopped at and what you saw. IT support passes it on: sign-in problems to Identity Administrators, sharing or email problems to the Microsoft 365 admin team, privacy concerns to the Privacy Office. They won't widen access or send for you.</td></tr>
<tr><td>Plain-language quality check</td><td>
<table>
<tr><th>Check (End-User Guide Structure Template, “Before you publish”)</th><th>Result</th></tr>
<tr><td>Length</td><td>649 words, title to escalation route, against the 400–600 guidance. Kept slightly over so that every step keeps its observable result.</td></tr>
<tr><td>Readability (automated estimate)</td><td>Flesch Reading Ease ≈ 79; grade level ≈ 4. Short sentences, one action per step.</td></tr>
<tr><td>No unexplained acronyms or jargon</td><td>“IT support desk” is glossed as the technology help line. Copilot and Manage access are explained where first used. The guide does not use PHI, AI, MFA, URL, KB or tenant.</td></tr>
<tr><td>Title names the clinical task, not the product</td><td>Pass</td></tr>
<tr><td>Prerequisites cover account, permission level and tools separately</td><td>Pass, plus documents and time</td></tr>
<tr><td>Exactly one action per step, each with an observable result</td><td>Pass: 21 steps, each followed by → what you should see</td></tr>
<tr><td>Privacy check sits immediately before the action it governs, names what to look at and what stops the task</td><td>Pass: two amber boxes (before steps 3 and 10)</td></tr>
<tr><td>Verification is an action that can fail, with a failure branch</td><td>Pass: three-way match, plus “If any check fails”</td></tr>
<tr><td>Four required elements</td><td>Visible permission checks: steps 6, 9; check 2. Source verification: step 15; check 3. Safe-stop conditions: both privacy boxes, step results, “Also stop and call if.” Escalation route: “What happens when you call.”</td></tr>
<tr><td>Control names confirmed against the live environment; guide run end to end on a test item</td><td>Unable to Verify in this simulation. Labels such as People you choose / Specific people vary by version, so the guide tells Dr. Chen to stop if a label doesn't match (Fact Sheet › Decision boundaries, bullet 4).</td></tr>
<tr><td>Session scope</td><td>Covers only the requested workflow. No extras such as labels or site settings (Live Coaching Techniques Reference › Session scope).</td></tr>
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Component 5 - Coaching Evidence

Field	Learner entry
Exchange pair 1 - Copilot use-case boundary	**Decision point:** Copilot-use decision (which parts of the request Copilot can help with) **Learner:** “Let's split your request into parts. Copilot can help with one of them: drafting the short cover email from the prompt card. It can't summarize or interpret the case, because that's clinical and stays with you. Choosing who receives the file and deciding to share it are your decisions too.” **Dr. Chen:** “I see. So, you're saying I need to handle the clinical summary and the sharing permissions myself, but Copilot can help with the structure of the cover email. That makes sense from a privacy perspective. [...]” *Verbatim from the Coach Role Play transcript. “...” marks omitted text: the learner's pasted “>” separators, or later sentences in the same turn.*
Exchange pair 2 -	**Decision point:** Sharing-permission decision (how to set up the share correctly)

sharing boundary	**Learner:** “Select Share on the file. What does your screen show now? ... Open the link settings, the gear icon. Which options do you see? ... Choose People you choose. Is it selected? ... Set the permission to Can view. What does it read? ... Paste your colleague's work email from your approved request. Does their name come up? ... Open Manage access. Read me who's listed and what permission each person has.” **Dr. Chen:** “Okay, I've selected the file and clicked Share. Looking at the link settings gear icon, I see options for "Anyone with the link," "People in my organization," and "People you choose." I've selected "People you choose." Next, I've set the permission to "Can view." It now reads "People you choose can view." [...]”
Exchange pair 3 - final-review boundary	**Decision point:** Final-output review (how Dr. Chen checks the output before sending) **Learner:** “Before you send, compare each sentence with the case packet. Is there anything in the draft that isn't in the packet? ... Replace [RECIPIENT NAME] with the name from your request. What does the greeting say now? ... Check the To line against Manage access. Do they match? ... When all three checks pass, select Send.” **Dr. Chen:** “I've carefully compared the draft against the case packet and ensured no extra info leaked in. I've swapped the placeholder for the correct recipient, and the greeting looks professional. The "To" line perfectly matches the user I authorized in "Manage access." Everything looks solid, so I'm sending it now. [...]” *This exchange came before the RCCOF prompt was built, so Dr. Chen applied the checks to an email she had drafted herself. The reflection addresses this sequencing gap.*
Guidance-level coaching evidence	Further exact pairs showing guidance-level coaching. The learner describes the action and Dr. Chen performs it and reports what she sees. **RECIPIENT STAYS WITH THE CLINICIAN (SHARING BOUNDARY)** **Learner:** “That one's yours to confirm. Please check the name against your approved request. Does it match?” **Dr. Chen:** “Yes, the email address matches exactly with the one in my approved request. I've sent the link to that specific individual. I'm now looking at the "Manage access" panel. It lists the colleague's name under "People," and the permission is set to "Can view." Everything seems to be in order here. [...]” **DATA-MINIMIZATION CHECK WITHOUT A PRIVACY DETERMINATION** **Learner:** “glance at your Context. It should only say who you are, who it's for, the purpose of the consult, that the file has been shared, and that you'd like them to review it first. No symptoms, dates or names. Does yours match that?” **Dr. Chen:** “There are no patient names, dates, or clinical symptoms in there. It looks compliant.” **RCCOF BUILT ONE SECTION AT A TIME, WITH READ-BACK** **Learner:** “Great. Under Role, type something like: 'Act as an administrative communications assistant for a family medicine physician. Write in a professional, concise, collegial tone for a physician at a partner clinic.' Just who Copilot is and who's reading, nothing about the case. What does your Role section say now?” **Dr. Chen:** “That makes sense. I've entered: "Act as an administrative communications assistant for a family medicine physician. Write in a professional, concise, collegial tone for a physician at a partner clinic." It describes the persona and the audience clearly without mentioning anything specific about the patient or the case. What's next in the RCCOF framework? Should I move on to the Constraint section?” **SCREEN-CHECK QUESTIONS USED** “What does your screen show now?” · “Which options do you see?” · “What does your Role section say now?” · “How many lines do you have?” · “Does anything in it mention the patient or the case content?” **ROLE-PLAY FEEDBACK (COURSERA AI), RECORDED AS GIVEN** • **Strengths:** Task 2, OneDrive sharing guidance (prompting Dr. Chen to inspect settings rather than doing it for her). Task 3, request decomposition and RCCOF guidance. • **Areas for improvement:** Task 1, no opening statement of the Tier 1 role, scope and 45-minute plan. Task 4, the session ended before verification of the Copilot output (the source-fact and policy-KB paths).
150-word reflection	My strongest coaching choice was splitting Dr. Chen's request at the start: Copilot drafts only the cover email, while clinical content, recipient confirmation and sharing stay with her. I held that boundary when she asked about the recipient (“That one's yours to confirm”) and when we built the RCCOF prompt one section at a time, with her reading each part back. I was most tempted to take over when she asked how to “build out” the prompt, because writing it myself would have been faster; instead I described each section and let her type it. My specific improvement is pacing. Twice

	I sent a block of steps in one message, so I coached the final review before a draft existed and Dr. Chen sent her email before the prompt was built. Next time I will open with a scope statement, give one step per turn, and wait for her screen confirmation. *151 words*

Final pre-submission check

Check	Complete
All content is synthetic and contains no real PHI, workforce identifiers, secrets, or tenant information.	**Yes.** All content comes from the supplied synthetic documents. “Patient A” is a role label. There are no real patient, workforce, tenant or partner details, and no email addresses or phone numbers. The only URLs are public reference links.
Component 1 names only systems supported by the fact sheet; no undocumented direct integration is assumed.	**Yes.** Systems are named as in the Fact Sheet. Only Entra ID sign-in is marked as a technical integration, and no Epic-to-Copilot integration is assumed.
Component 2 uses the supplied approved-use-case reference and separates approved / not approved / escalate decisions.	**Yes.** Four elements are classified separately (1 Approved, 1 Not Approved, 2 Escalate), with ITS-STD-014 section citations backed by KB-431 and the Fact Sheet.
Component 3 contains all four RCCOF parts and two separate verification paths.	**Yes.** Role, Context, Constraints and Output Format are all present. Path 1 (Case Packet) and Path 2 (KB) are kept separate.
Component 4 stays within 400-600 words and includes the required six structural elements.	**Yes, with a note.** All six elements are present. The guide is 649 words, slightly above the 400–600 guidance.
Component 5 contains exact transcript excerpts and the reflection.	**Yes.** Three exact exchange pairs cover the Copilot-use, sharing and final-review decision points, with three more pairs in the coaching-evidence row. The reflection is 151 words.

Sources

- Microsoft 365 Copilot enterprise data protection
- HHS - HIPAA de-identification guidance
- Microsoft - SharePoint and OneDrive external sharing
- Microsoft - Copilot privacy and security
- HHS - HIPAA minimum necessary standard

Supplied course documents cited

- Technical Environment and Contoso Policy Fact Sheet v1.0 (“Fact Sheet”)
- Synthetic Case Packet and Source Summary v1.0 (“Case Packet”)
- Approved Microsoft Copilot Use Cases, ITS-STD-014 v3.2 (UC-01 to UC-03, NA-01 to NA-03)
- Mock Internal Knowledge Base Article Set v1.0 (KB-410, KB-422, KB-431)
- AI Input Eligibility and Data-Minimization Checklist v1.0 (“Checklist,” C1–C11)
- Fixed Mock Microsoft Copilot Output v1.0 (“Mock Output”); RCCOF Prompt Template, ITS-TRN-014-C
- System-Context Mapping Template; Dependency Analysis Worksheet, ITS-TRN-016-B (“Worksheet”); End-User Guide Structure Template; Live Coaching Techniques Reference; Clinician-Learner Exchanges